ST.JOHN`S MEDICAL COLLEGE HOSPITAL

SARJAPURA ROAD, BANGALORE - 560034

Enquiry no: - 22065000 Appointment no: - 22065003

CASE SUMMARY & DISCHARGE RECORD

DEPARTMENT OF PAEDIATRIC NEPHROLOGY

Ms. THANUSHREE A
13 Years / Female
KARNATAKA (KA)

UHID
4259460

ENCOUNTER NO
902306020113

IP NR.
206231625824

DATE OF ADMISSION
02 JUN 2023

WARD/ROOM
PAEDIATRIC NEPHRO COMMON WARD - B BLOCK / 2005-F

UNIT
PAEDNEPH1

DATE OF DISCHARGE
07 JUN 2023

CONSULTANTS : PEDIATRIC NEPHROLOGY OPD DAYS (9am - 1pm) Dr. Anil Vasudevan Prof & Head Tue / Sat Dr. Arpana Iyengar Professor Thur / Sat (Tue - Evening special OPD) Dr. Priya Pais Assoc Prof Tue / Thur Dr. Nivedita Asst Prof Wed / Sat Monday: CKD and Transplant Clinics at 11am - 4pm (Unit of Hope Building) Wednesdays: NUC: 2pm - 4pm OPD days are Tuesday/Thursday & Saturday (New/Silver Jubilee OPD) between 9am - 12.30pm.

ADMISSION DIAGNOSIS : SLE , RENAL BIOPSY - CLASS 1[LUPUS NEPHRITIS]
APS NEPHROPATHY WITH CORTICAL NECROSIS
ON MAINATAINCE DIALYSIS , ACCESS RT IJV
COMORBIDITIES :
ANEMIA , MBD , STAGE -2 HTN , ACIDOSIS AND PROXIMAL MYOPATHY

PRESENTING COMPLAINTS : Admitted for the Rituximab

HISTORY : 14 yr old , Tanushree , was admitted in PHO , i/vo ITP failure to MMF and was admitted foir the Rituximab . child was diagnosed as New Onset ITP [April 19] , had refractory to the IV IG , child was started with MMF[1250mg/m2][500mg-0-750mg] and Prednisolone x 2 weeks .secondary work up – negative [ANA- neg, viral markers- negative]While child continued with same dosage and remained asymptomatic . July 21 , MMF 500mg bd was decreased . Oct 21 , suspected Evans Syndrome i/v/o pancytopenia [hb-7.8,TLC- 4717,P-3000], child was evaluated for the secondary work up , and was found to have , ANA
-2+ , DCT- neg , dose of MMF was increased with prednisolone [2.56 lakh , hb-8.3,TLC-19.93] later child continued with same dosage of MMF . child continued with same dosage , in month of 17 th April admitted i/v/o menorhagia and was treated symptomatically . however

, past 4 days child had persistent thrombocytopenia [plt- <5k] received MPS [30mg/kg] over 5days , i/v/o refractory , child had admitted for the , Rituximab [29/04/23] .

PAST HISTORY : as mentioned in the hsitory

FAMILY HISTORY : no h/o similar complaints in the family

DEVELOPMENT HISTORY / DIET HISTORY / IMMUNIZATION : devleopmentally nornal child
immunsied as per national immunsiation schedule
optional vaccines not recived .

ON EXAMINATION : child alert and active
pallor + , no icturus and lymphadenopathy
PP- 98
BP- 110/70
CRT<3SEC
TEMP- AFEBRILE
SAO2- 98% at room air

CVS- S1 and S2 , NO GALLOP
RS- B/L AE+
P/A - SOFT NO ORGANOMEGALY
CNS- NAD

DISCUSSION : 14 yr old , child admitted with following issues :
SLE with APLA with APS Nephropathy : child was diagnosed as New Onset ITP [April 19] , had refractory to the IV IG , child was started with MMF[1250mg/m2][500mg-0-750mg] and Prednisolone x 2 weeks .secondary work up – negative [ANA- neg, viral markers- negative]
While child continued with same dosage and remained asymptomatic . July 21 , MMF 500mg bd was decreased . Oct 21 , suspected Evans Syndrome i/v/o pancytopenia [hb-7.8,TLC- 4717,P-3000], child was evaluated for the secondary work up , and was found to have , ANA
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, past 4 days child had persistent thrombocytopenia [plt- <5k] received MPS [30mg/kg] over 5days , i/v/o refractory , child had received , Rituximab [29/04/23] of 100mg ,during rituximab child had worsening of edema , with severe pulmonary edema with oligoanuria ,
required haemodialysis , was shifted in PICU . child was revelauted with secondary work up , ANA – 2+ , immunolines – negative , C-3[17.3] , C4[8.35] low , secondary viral markers –negative , PS- done s/o NCNC , reticulocyte- 2.36 , LDH-271, D dimer -271 ,APLA

+[Anticardiolipin IgG- 23 , B2 Glycoprotein – IgG56 , , B2 Glycoprotein – IgM20,]Lupus anticoagulant + [02/05/23], Immunoline work up- negative , urine routine – albumin-2+ ,R- 158,W-101 , with serum albumin- 3.6, serum creatinine -4.5, peak -6.07, with underlying
fungal infection , Galactomannan-2.5 , for which child received iv amphotericin and later shifted to oral fluconazole . child persistent to remain dialysis dependent and on alternate dialysis . child was evaluated , USG , s/o RK-10.2 X 1.2 cm and LK- 10.2 cmx 1.1 cm with CMD maintained with garde 1 parenchymal changes . Renal biopsy done s/o i/v/o APS nephropathy with APLA , s/o cortico medullary tissue with cortical necrosis noted involving 40% core . interlobular artery evident insections s/o organised thrombi with fibrin deposition in the vessel wall . glomeruli adjacent to the thrombosis vessel suggestive of sclerosis with wrinkling of basemnet membrane . the viable glomeruli in renal tissue are unremarkable . no evdence of endocapillary hypercellularity /fibroid necrosis /cresecnt seen in the viable glomeruli. the tubules in viable Rheumatology opinion was taken and advised for the inj Rituximab x 4 doses and plan to consider restarting MPA v/s Azathioprine . However , child has complteted 3 doses of rituximab with dexamtheasone of 40mg/m2 , tolertaed and unevenful .

AKD : child was started dialyssi from the 28/04/23 access RT IJV , child continued to remain oliuria [u/o- 300-500 ml] with once during hospital stay had urine output- 900ml /day . i/v/o dilaysis depenedent long term plan was to consider , AVF v/s CAPD ,both the options was provided to the parents . i//v/o high risk of thrombosis , plan was to defer the AVF and CAPD was differed i/v/o finanacial constraint . so plan was to consider , permcath and continue haemodilaysis at thier native place [Tumkur] on 3 times /weekly . plan to continue anticoagulants Aspirin and Enoxaparin duirng non dialysis period .

Anemia - hb- , on iron and folic acid supplements and inj EPO [ ] has recived PRBC tansfusion , plan to contniue supplements .

Hypertension - bp well controlled between 50 - 90 th centile , and end organ damage evaluation , opthalmology - grade 1 hypertensive cahnges and 2d echo - conc LVH , EF-

MBD- calcium - phosphorus , vitamin d- , PTH - , on calcium supplements and vitamin d supplements .

Acidosis :on alternate day haemodialysis

Extra renal manefestations :

Proximal Myopathy : child had proximal myopathy , secondary attributed to the prolonged steriod . child was evalauted for the same , MRI spine was done s/o disc bulge noted at the L5S1 , neurology was reviwed .

Major Depression : During hospital , stay child was depressed because of the underlying disease illness and the long term requiremnet of dialysis . child was reviewd by pediatric counselling and was advised for the regular counselling at the OPD basis.

Prepared By :	**Checked By :**
Name : _____	Name : _____
KMC NO.: _____	KMC NO.: _____
Date & Time: _____	Date & Time: _____
Summary Recieved by: _____	Signature: _____
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